

CALMILLE

Tablet 1 mg

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Each tablet contains 1.0 mg of Alprazolam

Description: Tablet, round, shallow convex, plain on side I, breakline on side II, pink color, diameter 5.80 – 6.20 mm, thickness 2.40 – 3.00 mm.

Pharmacology

Pharmacodynamics :

Alprazolam, like other benzodiazepines, has a high affinity for the benzodiazepine binding site in brain. It facilitates the inhibitory neurotransmitter action of gamma aminobutyric acid, which mediates both pre and post synaptic inhibition in the central nervous system (CNS).

Pharmacokinetics :

Following oral administration, peak concentrations of Alprazolam Tablets in plasma occur in one to two hours following administration. The mean half-life of Alprazolam is 12-15 hours.

Alprazolam is mainly oxidized. The predominant metabolites are alpha-hydroxy-alprazolam and a benzophenone derived from Alprazolam. Plasma levels of these metabolites are extremely low. The biological activity of alpha-hydroxy-alprazolam is approximately one-half that of Alprazolam. Their half-lives appear to be of the same order of magnitude as that of Alprazolam. The benzophenone metabolite is essentially inactive. Alprazolam and its metabolites are excreted primarily in the urine.

In vitro, Alprazolam is bound (80%) to human serum protein.

Indication

Calmille Tablets are indicated for the treatment of:

- **Anxiety states (anxiety neuroses)**

Symptoms which occur in such patients include anxiety, tension, agitation, insomnia, apprehension, irritability and/or autonomic hyperactivity resulting in a variety of somatic complaints.

- **Mixed anxiety-depression**

Symptoms of both anxiety and depression occur simultaneously in such patients.

- **Neurotic or reactive depression**

Such patients primarily exhibit a depressed mood or a pervasive loss of interest or pleasure. Symptoms of anxiety, psychomotor agitation and insomnia are usually present. Other characteristics include appetite disturbances, changes in weight, somatic complaints, cognitive disturbances, decreased energy, feeling of worthlessness or guilt, or thoughts of death or suicide.

- **Anxiety states, mixed anxiety-depression, or depression** associated with other diseases, such as the chronic phase of alcohol withdrawal and functional or organic disease, particularly certain gastrointestinal, cardiovascular, or dermatological disorders.

- **Panic related disorders**

Calmille is indicated in the treatment of panic disorder with or without some phobic avoidance. Calmille is also indicated for the blocking or attenuation of panic attacks and phobias in patients who have agoraphobia with panic attacks.

Dosage and Administration

The optimum dosage should be individualized based upon the severity of the symptoms and individual patient response. In patients who require higher doses, dosage should be increased cautiously to avoid adverse effects. In general, patients who have not previously received psychotropic medications will require somewhat lower doses than those previously treated with minor tranquilizers, antidepressants, or hypnotics. It is recommended that the general principle of using the lowest effective dose be followed in elderly or debilitated patients to preclude the development of oversedation or ataxia.

Duration of Treatment

The overall duration of treatment should not be more than 8-12 weeks, including a tapering off process.

Discontinuation of Treatment

To discontinue alprazolam treatment, the dosage should be reduced slowly in keeping with good medical practice. It is suggested that the daily dosage of alprazolam be decreased by not more than 0.5 mg every 3 days. Some patients may require an even slower dosage reduction (see Warnings and Precautions).

Pediatric Use

Safety and efficacy have not been established in children under 18 years of age.

Posology

Alprazolam IR Tablets Indication or Population	Usual Starting Dose (if side effects occur, dose should be lowered)	Usual Dose Range
Anxiety	0.75 to 1.5 mg given in divided doses.	0.5 to 4.0 mg daily, given in divided doses.
Depression	1.5 mg daily given in divided doses.	1.5 to 4.5 mg daily, given in divided doses.
Panic-related Disorders	0.5 to 1.0 mg, given at bedtime or 0.5 mg three times daily.	Dose should be adjusted to patient response with increments not greater than 1mg/day every 3 or 4 days. Additional doses can be added until a schedule of three or four times daily is achieved. The mean dose in a large multi-clinic study was 5.7 ±2.27 mg, with occasional patients requiring a maximum of 10mg/day.
Geriatric Patients	0.5 to 0.75 mg daily given in divided doses	0.5 to 0.75 mg daily; may be gradually increased if needed and tolerated.

Contraindication

Patients who have hypersensitivity to benzodiazepines, alprazolam, or to any of the excipients of these products' formulation. Benzodiazepines are also contraindicated in patients with acute narrow-angle glaucoma, myasthenia gravis, severe respiratory insufficiency, sleep apnoea syndrome, severe hepatic insufficiency.

Warning and Precautions

Renal and hepatic impairment

Caution is recommended when treating patients with impaired renal function or mild to moderate hepatic insufficiency.

Depression/anxiety

In patients presenting with major depression or anxiety associated with depression benzodiazepines and benzodiazepine-like agents should not be prescribed alone to treat depression as they may precipitate or increase the risk of suicide. Therefore alprazolam should be used with caution and the prescription size should be limited in patients with signs and symptoms of a depressive disorder or suicidal tendencies.

Paediatric population

Safety and efficacy of alprazolam have not been established in children and adolescents below the age of 18 years; therefore use of alprazolam is not recommended.

Elderly patients

Benzodiazepines and related products should be used with caution in elderly, due to the risk of sedation and / or musculoskeletal weakness that can promote falls, often with serious consequences in this population. It is recommended that general principle of using the lowest effective dose to be followed in elderly and / or debilitated patients to preclude development of ataxia or over-sedation (see section Recommended Dose). A lower dose is also recommended for patients with chronic respiratory insufficiency due to risk of respiratory depression. Benzodiazepines should be used with extreme caution in patients with a history of alcohol or drug abuse (see section Interactions with Other Medicaments).

Dependence

Use of benzodiazepines may lead to the development of physical and psychic dependence upon these products. The risk of dependence increases with dose and duration of treatment; it is also greater in patients with a history of alcohol and drug abuse. Pharmacodependency may occur at therapeutic doses and/or in patients with no individualised risk factor. There is an increased risk of pharmacodependency with the combined use of several benzodiazepines regardless of the anxiolytic or hypnotic indication. Cases of abuse have also been reported.

Withdrawal symptoms: Once physical dependence has developed, abrupt termination of treatment will be accompanied by withdrawal symptoms. These may consist of headaches, muscle pain, extreme anxiety, tension, restlessness, confusion, irritability and insomnia. In severe cases the following symptoms may occur: derealization, depersonalisation, hyperacusis, numbness and tingling of the extremities, hypersensitivity to light, noise and physical contact, hallucinations or epileptic seizures (see section Recommended Dose).

During discontinuation of alprazolam treatment, the dosage should be reduced slowly in keeping with good medical practice. It is suggested that the daily dosage of alprazolam be decreased by no more than 0.5 mg every three days. Some patients may require even slower dosage reduction.

Rebound insomnia and anxiety: a transient syndrome whereby the symptoms that led to treatment with a benzodiazepine recur in an enhanced form may occur on withdrawal of treatment. It may be accompanied by other reactions including mood changes, anxiety or sleep disturbances and restlessness. Since the risk of withdrawal phenomena/rebound phenomena is greater after abrupt discontinuation of treatment, it is recommended that the dosage be decreased gradually by no more than 0.5 mg every three days. Some patients may require an even slower dose reduction (see section Recommended Dose).

Duration of treatment

The duration of treatment should be as short as possible (see section Recommended Dose) depending on the indication, but should not exceed eight to twelve weeks including tapering off process. Extension beyond these periods should not take place without re-evaluation of the situation. It may be useful to inform the patient when treatment is started that it will be of limited duration and to explain precisely how the dosage will be progressively decreased. Moreover it is important that the patient should be aware of the possibility of rebound phenomena, thereby minimising anxiety over such symptoms should they occur while the medicinal product is being discontinued. There are indications, that in the case of benzodiazepines with a short duration of action, withdrawal phenomena can become manifest within the dosage interval, especially when the dosage is high. When benzodiazepines with a long duration of action are being used it is important to warn against changing to a benzodiazepine with a short duration of action, as withdrawal symptoms may develop.

Amnesia

Benzodiazepines may induce anterograde amnesia. The condition occurs most often several hours after ingesting the product and therefore to reduce the risk patients should ensure that they will be able to have uninterrupted sleep of 7-8 hours (see section Side Effects).

Psychiatric and paradoxical reactions

Reactions like restlessness, agitation, irritability, aggressiveness, delusion, rages, nightmares, hallucinations, psychoses, inappropriate behaviour and other adverse behavioural effects are known to occur when using benzodiazepines. Should this occur, use of the medicinal product should be discontinued.

They are more likely to occur in children and the elderly.

Tolerance

Some loss of efficacy to the hypnotic effects of benzodiazepines may develop after repeated use for a few weeks.

Episodes of hypomania and mania have been reported in association with the use of alprazolam in patients with depression.

Benzodiazepines are not recommended for the primary treatment of psychotic illness.

Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Effects on ability to drive and use machines

Sedation, amnesia, impaired concentration and impaired muscle function may adversely affect the ability to drive or use machines. If insufficient sleep occurs, the likelihood of impaired alertness may be increased (see section Interactions with Other Medicaments).

These effects are potentiated by alcohol (see section Interactions with Other Medicaments).

Patients should be cautioned about operating motor vehicles or engaging in other dangerous activities while taking Alprazolam.

This medicine can impair cognitive function and can affect ability to drive safely.

- Anaphylaxis (severe allergic reaction) and angioedema (severe facial swelling) which can occur as early as the first time the product is taken
- Complex sleep – related behaviors which may include sleep driving, making phone calls, preparing and eating food while a sleep

Pregnancy:

The data concerning teratogenicity and effects on postnatal development and behavior following benzodiazepine treatment are inconsistent. A large amount of data based on cohort studies that first trimester exposure to benzodiazepine is not associated with an increase in the risk of major malformation. However, some early case-control epidemiological studies have found a two fold increased risk of oral clefts.

Benzodiazepines including alprazolam should only be used during pregnancy or lactation if considered essential by the physician. Animal studies with benzodiazepines have shown minor effects on the foetus while a few studies have reported late behavioural disturbance in offspring exposed in utero.

Benzodiazepine treatment at high dose, during the second and/or the third trimester of pregnancy, has revealed a decrease of foetal active movements and a variability of foetal cardiac rhythm.

When treatment has to be administered for medical reasons during the last part of pregnancy, even at low doses, floppy infant syndrome such as axial hypotonia, sucking troubles leading to a poor weight gain may be observed. These signs are reversible but they may last from 1 up to 3 weeks, according to the half-life of the product. At high doses, respiratory depression or apnoea and hypothermia in newborn may appear. Moreover, neonatal withdrawal symptoms with hyperexcitability, agitation ad tremor may be observed a few days after birth, even if no floppy infant syndrome is observed. The apparition of withdrawal symptoms after birth depends on the half life of the substance.

Alprazolam should not be used during pregnancy unless the clinical condition of the woman requires treatment with alprazolam. If alprazolam is used during pregnancy, or if the patient becomes pregnant while taking alprazolam, the patient should be apprised of the potential hazard to the foetus. If alprazolam treatment is necessary during last part of pregnancy, or during labour high doses should be avoided and withdrawal symptoms and/or floppy infant syndrome should be monitored in newborn.

Breast-feeding :

Alprazolam is excreted in breast milk at low level. However, alprazolam is not recommended during breast-feeding.

Drug Interaction

Benzodiazepines produce an additive effect when co-administered with alcohol or other CNS depressants. Concomitant intake with alcohol is not recommended. Alprazolam should be used with caution when combined with CNS depressants. Enhancement of the central depressive effect may occur in cases of concomitant use with antipsychotics (neuroleptics), hypnotics, anxiolytics/sedatives, antidepressant agents, narcotic analgesics, anti-epileptic drugs, anaesthetics and sedative antihistamines. In the case of narcotic analgesics enhancement of the euphoria may also occur leading to an increase in psychic dependence.Pharmacokinetic interactions can occur when alprazolam is administered along with drugs that interfere with its metabolism.

CYP3A Inhibitors

Compounds that inhibit certain hepatic enzymes (particularly cytochrome P450 3A4) may increase the concentration of alprazolam and enhance its activity. Data from clinical studies with alprazolam, in-vitro studies with alprazolam and clinical studies with drugs metabolised similarly to alprazolam provide evidence for varying degrees of interaction and possible interaction with alprazolam for a number of drugs. Based on the degree of interaction and the type of data available, the following recommendations are made:

- The co-administration of alprazolam with ketoconazole, itraconazole, or other azole-type antifungals is not recommended.
- The co-administration of nefazodone or fluvoxamine increases the AUC of alprazolam by approximately 2-fold. Caution and consideration of dose reduction is recommended when alprazolam is co-administered with nefazodone, fluvoxamine and cimetidine.
- Caution is recommended when alprazolam is co-administered with fluoxetine, propoxyphene, oral contraceptives, sertraline, diltiazem, or macrolide antibiotics such as erythromycin, clarithromycin and troleandomycin

CYP3A4 Inducers

Since alprazolam is metabolized by CYP3A4, inducers of this enzyme may enhance the metabolism of alprazolam. Interactions involving HIV protease inhibitors (e.g. ritonavir) and alprazolam are complex and time dependent. Short term, low doses of ritonavir resulted in a large impairment of alprazolam clearance, prolonged its elimination half-life and enhanced clinical effects. However, upon extended exposure to ritonavir, CYP3A induction offset this inhibition. This interaction will require a dose-adjustment or discontinuation of alprazolam.

Digoxin

Increased digoxin concentrations have been reported when alprazolam was given, especially in elderly (>65 years of age). Patients who receive alprazolam and digoxin should therefore be monitored for signs and symptoms related to digoxin toxicity.

Side Effects

Adverse events, if they occur, are generally observed at the beginning of therapy and usually disappear upon continued medication or decreased dosage.

<i>MeDRA System Organ Class</i>	<i>Frequency</i>	<i>Side Effects</i>
Endocrine disorders	Not known	Hyperprolactinaemia
Metabolism and nutrition disorders	Common	Decreased appetite
Psychiatric disorders	Very common	Depression
	Common	Confusional state, disorientation, libido decreased, anxiety, insomnia, nervousness, libido increased
	Uncommon	Mania, (see section Warning and Precautions) hallucination, anger, agitation
	Not known	Hypomania, aggression, hostility, thinking abnormal,psychomotor hyperactivity
Nervous system disorders	Very common	Sedation, somnolence, ataxia, memory impairment, dysarthria, dizziness, headache
	Common	Balance disorder, coordination abnormal, disturbance in attention, hypersomnia, lethargy, tremor
	Uncommon	Amnesia
	Not known	Autonomic nervous system imbalance, dystonia
Eye disorders	Common	Vision blurred
Gastrointestinal disorders	Very common	Constipation, dry mouth
	Common	Nausea
	Not known	Gastrointestinal disorder
Hepatobiliary disorders	Not known	Hepatitis, hepatic function abnormal, jaundice
Skin and subcutaneous tissue disorders	Common	Dermatitis
	Not known	Angioedema, photosensitivity reaction
Musculoskeletal and connective tissue disorders	Uncommon	Muscular weakness
Renal and urinary disorders	Uncommon	Incontinence
	Not known	Urinary retention
Reproductive system and breast disorders	Common	Sexual dysfunction
	Uncommon	Menstruation irregular
General disorders and administration site conditions	Very common	Fatigue, irritability
	Not Known	Oedema peripheral
Investigations	Common	Weight increased, weight decreased
	Not Known	Intraocular pressure increased

Withdrawal symptoms have occurred following rapid decrease or abrupt discontinuance of benzodiazepines including alprazolam. These can range from mild dysphoria and insomnia to a major syndrome, which may include abdominal and muscle cramps, vomiting, sweating, tremor and convulsions. In addition, withdrawal seizures have occurred upon rapid decrease or abrupt discontinuation of therapy with alprazolam.

Amnesia

Anterograde amnesia may occur at therapeutic dosages, the risk increasing at higher dosages. Amnesic effects may be associated with inappropriate behaviour (see section Warning and Precautions).

Depression

Pre-existing depression may be unmasked during benzodiazepine use.

Psychiatric and paradoxical reactions

Reactions like restlessness, agitation, irritability, aggressiveness, delusion, rages, nightmares, hallucinations, psychoses, inappropriate behaviour and other adverse behavioural effects are known to occur when using benzodiazepines or benzodiazepine-like agents. They may be quite severe with this product. They are more likely to occur in children and the elderly. In many of the spontaneous case reports of adverse behavioural effects, patients were receiving other CNS drugs concomitantly and/or were described as having underlying psychiatric conditions. Patients who have borderline personality disorder, a prior history of violent or aggressive behaviour, or alcohol or substance abuse may be at risk of such events. Instances of irritability, hostility and intrusive thoughts have been reported during discontinuance of alprazolam in patients with post-traumatic stress disorder.

Dependence

Use (even at therapeutic doses) may lead to the development of physical dependence: discontinuation of the therapy may result in withdrawal or rebound phenomena. Psychic dependence may occur. Abuse of benzodiazepines has been reported.

Overdose

As with other benzodiazepines, overdose should not present a threat to life unless combined with other CNS depressants (including alcohol). In the management of overdose with any medicinal product, it should be borne in mind that multiple agents have been taken. Following overdose with oral benzodiazepines, vomiting may be induced (within 1 hour) if the patient is conscious or gastric lavage undertaken with the airway protected if the patient is unconscious. If there is no advantage in emptying the stomach, activated charcoal should be given to reduce absorption. Special attention should be paid to respiratory and cardiovascular functions in intensive care. Overdose of benzodiazepines is usually manifested by degrees of central nervous system depression ranging from drowsiness to coma. In mild cases, symptoms include drowsiness, mental confusion and lethargy, in more serious cases, symptoms may include ataxia, hypotonia, hypotension, respiratory depression, rarely coma and very rarely death. Flumazenil may be useful as an antidote.

Presentations

Calmille Tablet 1 mg

Box of 6 strips x 10 tablets
Reg. No. : DPL0033501110C1
Shelf Life: 2 Years

Storage Condition :
Do not store above 30°C, store in the original package.

ON MEDICAL PRESCRIPTION ONLY

Product Registration Holder :
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